NCCP 00642 · VERSION 2

Your treatment, explained

Progressive, unresectable or metastatic, well-differentiated G1/G2 somatostatin-receptor-positive gastroenteropancreatic NET under NCCP 00642a. · NCCP 00642 v2 · Document revised 5 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Lutetium-177 oxodotreotide (Lutathera / PRRT)

Four planned administrations, usually every eight weeks

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Seek urgent help for sudden severe flushing with faintness, severe diarrhoea, wheeze or breathing trouble, especially in the first 48 hours; follow the hormone-crisis plan. Fever, bleeding, much less urine, persistent severe abdominal pain, muscle weakness or confusion needs prompt advice. Tell emergency staff when you received PRRT; radiation precautions must not delay urgent care.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp · smpc · hse-prrt

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
Preparation, treatment day and follow-up
Your 56-day timeline
Baseline and cycle-specific checks
Whole treatment, scans and checks
Tablets, visits and practical planning
Your PRRT course
Preparing for hospital and returning home
Blood tests, scans and follow-up
Urgent symptoms, hormone safety and precautions
Your Irish PRRT service plan in print
Understanding your particular NET
Making sense of tests and results
Hormone symptoms and day-to-day changes
Your overall plan and choices
Eating, energy, practical help and support
Supportive medicines
Understanding the evidence
Questions to ask
Sources

UNDERSTAND YOUR TREATMENT

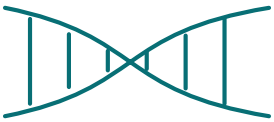
Targeted radiation and kidney protection

PRRT carries radiation to somatostatin receptors on NET cells to help control disease that cannot be removed or has spread. A specialist NET and nuclear-medicine team checks receptor imaging, organ function and your priorities before recommending it.

RECEPTOR-TARGETED RADIOPHARMACEUTICAL

Lutetium-177 oxodotreotide

Lutathera / 177Lu-DOTATATE / PRRT



Carries radiation to somatostatin receptor targets

Lutathera joins a receptor-binding molecule to the radioactive element lutetium-177. After infusion, it attaches to somatostatin receptors on suitable NET cells and delivers radiation nearby. The receptor scan helps select patients. Kidneys and blood-forming marrow can also receive radiation, which is why kidney protection and long-term blood monitoring matter.

SSTR target on NET cells → Radiolabelled medicine attaches → Radiation damages tumour cells

What to watch for

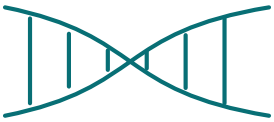
Keep all between-treatment and after-treatment blood tests; use your personalised radiation-safety and hormone-symptom plans.

Sources: nccp · smpc · hse-prrt

SUPPORTIVE INFUSION

Lysine / arginine amino acids

Kidney-protective infusion



Reduces kidney uptake of radioactivity

The amino-acid infusion reduces the amount of radioactive medicine retained in the kidneys. It is an essential part of kidney protection. It can cause nausea and affect potassium or fluid balance, so anti-sickness treatment, blood tests and observation are needed.

Amino acids infused → Kidney uptake reduced → Kidney radiation reduced

What to watch for

Tell staff promptly about nausea, palpitations, weakness, breathlessness or swelling during infusion.

Sources: nccp · smpc

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Energy / wellbeing

PRRT

Tiredness may last beyond the treatment day. Severe weakness, confusion, seizure or collapse needs urgent assessment.

3 Heart / fluids / potassium

Amino acids / hormone effects

Potassium or fluid changes can affect the heart. Report palpitations, new swelling or breathlessness promptly; severe symptoms are an emergency.

5 Liver

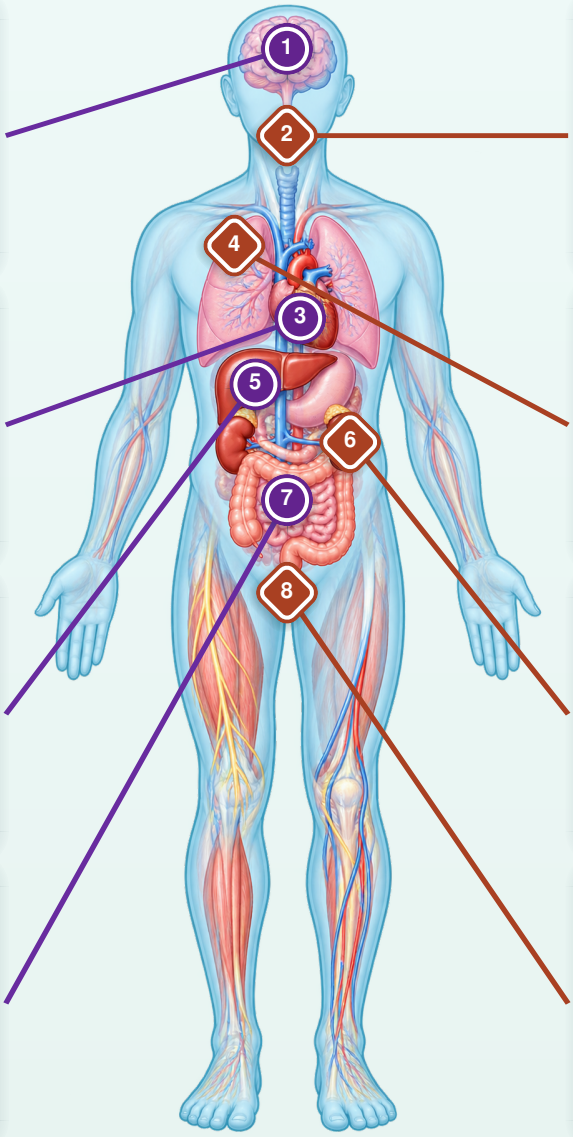
PRRT

Liver tests are monitored before, between and after treatments. Report yellow skin or eyes, dark urine or persistent severe abdominal pain.

7 Bowel / hormone symptoms

PRRT / NET

Abdominal pain, diarrhoea or sudden hormone release can occur. Severe flushing, faintness, diarrhoea or wheeze needs urgent advice.



2 Appetite / sickness

PRRT / amino acids

Nausea and vomiting can occur, especially around treatment. Tell staff early and follow the anti-sickness and fluid plan.

4 Bone marrow / blood

PRRT

Counts may fall during or after the course. Report fever, bruising or bleeding. Rare late marrow disorders require ongoing follow-up.

6 Kidneys / urine

PRRT

Amino acids reduce kidney exposure but do not remove risk. Keep renal blood tests and report much less urine or dehydration.

8 Fertility / pregnancy

PRRT

Discuss fertility before treatment. Avoid pregnancy using the written contraception plan; follow individual radiation precautions for close contacts.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

Preparation, treatment day and follow-up

T1–T4

Four planned PRRT administrations

T1 T2 T3 T4

Usually eight weeks between administrations, with blood tests every two weeks and a PRRT clinic review. The service confirms each date and any delay.

Four administrations are intended, not guaranteed. Blood results, recovery, specialist review and the service plan determine whether and when each can be given.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

PRRT tests before, between and after four intended administrations

This is the NCCP blood-test sequence with EMA potassium safety and the Irish PRRT service pathway. The post-infusion radiation-assessment scan is different from a response scan. The service confirms bookings, exact dates and any additional follow-up.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00642 v2 · current Irish Lutathera regimen · EMA Lutathera product information and patient leaflet · updated 14 August 2026 · HSE / Irish PRRT service: introductory patient information

THE WHOLE TREATMENT JOURNEY

Lutetium-177 oxodotreotide (Lutathera / PRRT): treatment and checks

Progressive, unresectable or metastatic, well-differentiated G1/G2 somatostatin-receptor-positive gastroenteropancreatic NET under NCCP 00642a.

Four administrations are planned, usually eight weeks apart. Lutathera is infused over about 30–40 minutes, with a four-hour kidney-protective amino-acid infusion and anti-sickness treatment. The visit is much longer than the Lutathera infusion; an overnight stay may be needed.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before the course	Before each administration	Between administrations	After the final dose	NET response / safety plan
Blood count / kidneys / liver NCCP 00642	Baseline tests	Once 2–4 weeks beforehand AND shortly before infusion	Every 2 weeks	Every 4 weeks for at least 3 months, then every 3 months for the following year	Additional tests or longer follow-up if abnormal
Potassium / renal clearance NCCP 00642	Assess renal function and electrolyte risks	Potassium before amino acids; renal function and suitability reviewed	Renal checks during therapy; earlier assessment if ill	Renal monitoring for at least the first year	Fluid and electrolyte plan individualised
Heart / pregnancy / hormones NCCP 00642	ECHO or MUGA; pregnancy testing where applicable; hormone plan	Review symptoms and any pregnancy possibility; SSA plan confirmed	Report hormone symptoms, especially during first 48 hours	Follow hormone symptoms and contraception plan	Additional heart tests if clinically indicated
Imaging / clinic NCCP 00642	SSTR-positive disease and baseline assessment	Specialist clearance for each treatment	PRRT clinic review; special scan(s) after each infusion for radiation assessment	Response and follow-up imaging as agreed	No universal mandatory tumour-marker or response PET interval
Radiation precautions NCCP 00642	Discuss household, travel, work and care needs	Receive written instructions for this administration	Observe each restriction until its own end date	Keep discharge information and tell other treating teams	Medical physicist confirms personal restrictions

What each check is for

1

Four planned PRRT administrations

T1–T4

PRRT carries radiation to somatostatin receptors on NET cells to help control disease that cannot be removed or has spread. A specialist NET and nuclear-medicine team checks receptor imaging, organ function and your priorities before recommending it.

Ask / act: Confirm the current prescription, tests and support medicines.

Sources: NCCP 00642

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Day 1 → Day 56: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Four administrations are planned, usually eight weeks apart. Lutathera is infused over about 30–40 minutes, with a four-hour kidney-protective amino-acid infusion and anti-sickness treatment. The visit is much longer than the Lutathera infusion; an overnight stay may be needed.

Treatment timing: displayed days 1–56

Treatment or check	Days in this window	What happens
PRRT and protective infusions	Day 1	Anti-sickness treatment, amino acids and Lutathera; admission / overnight plan confirmed.
Personal radiation-safety plan	Days 1–21	Restriction periods differ by activity and contact; follow your written end dates, not the band alone.
Fortnightly blood tests and PRRT review	Days 1–56	Blood count, kidneys and liver; dates are booked individually.
Assessment for next administration	Day 56	Usually eight weeks later; proceed only after specialist approval.

Four administrations, not four consecutive days: The intended administrations are usually eight weeks apart. Record each actual date and any change; delays do not authorise a catch-up treatment.

Follow-up extends beyond the fourth treatment: Blood counts, kidneys and liver require continued checks. Kidney injury and rare marrow disorders may appear later, so retain the follow-up schedule and report new symptoms.

Two kinds of scan: The Irish service describes special scans after each infusion to measure radiation delivered to organs. CT/MRI or other scans to assess tumour response follow the specialist plan.

Radiation safety has individual end dates: A contact restriction, work restriction and sleeping arrangement may finish on different dates. The wallet or calendar is a reminder; your written radiation-safety instructions control the details.

> **Why these bands stay broad:** The 56-day view shows a usual interval, not a guarantee of the next administration. The radiation band shows a possible period only; each restriction has its own written end date from the service. Symptoms and abnormal tests can persist after any band ends. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00642 v2 · current Irish Lutathera regimen ↗ · EMA Lutathera product information and patient leaflet · updated 14 August 2026 ↗ · HSE / Irish PRRT service: introductory patient information ↗

> **There are no guaranteed “safe days”.** Follow your oncology unit’s emergency instructions. Fever, shivering or suddenly feeling unwell needs urgent advice; do not wait for the next appointment. Call emergency services for collapse, severe breathing difficulty, chest pain or uncontrolled bleeding.

Complete timing and symptom notes

Day 1–2: Treatment and early observation

Plan a long hospital visit; the first treatment generally involves an overnight stay. Staff give anti-sickness medicine and kidney-protective amino acids. Hormone reactions are especially important in the first 48 hours. Before discharge, obtain written emergency contacts, SSA timing and radiation precautions.

Day 3–21: Recovery and individual precautions

Energy, appetite and bowel symptoms may change. Radiation precautions may last two to three weeks for some activities or contacts; exact restrictions come from the medical physicist. Attend blood tests every two weeks. Call for fever, bleeding, reduced urine, severe pain or sudden hormone symptoms.

Day 22–56: Tests and next-treatment decision

Continue fortnightly bloods and attend the PRRT review. A post-infusion scan checks radiation distribution; response scans follow a separate specialist plan. The team confirms whether and when the next administration is suitable. Do not treat an eight-week date as automatic clearance.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before admission

Confirm SSTR imaging, appointments, recent bloods, hormone medicines and pregnancy precautions. Tell the team about kidney/heart problems, incontinence, a catheter, childcare or close contact with pregnant people; these may change the practical plan.

STEP 2

Treatment day

The standard NCCP activity is 7.4 GBq by intravenous infusion, usually every eight weeks for four administrations; the specialist may alter, delay or stop treatment. Lutathera runs over approximately 30–40 minutes. A lysine/arginine amino-acid infusion starts 30 minutes beforehand and continues for four hours. An anti-sickness medicine is given at least 30 minutes before the amino acids. The first visit generally includes an overnight stay; later treatments may be day case or overnight. The service confirms your plan.

STEP 3

SSA injections: stop and restart

The NET and nuclear-medicine teams must coordinate the injection plan. Do not translate “about four weeks” into your own date: NCCP specifies 30 days for long-acting SSA and 24 hours for short-acting SSA before PRRT. Restart timing follows the written service plan; it can differ with day-case arrangements.

STEP 4

At home after treatment

Follow individual instructions on distance and time with other people, sleeping arrangements, toilets, body fluids, laundry, work and transport. Restrictions may extend to three weeks for some contacts. Contact the medical physicist before discharge if you cannot follow the plan at home.

STEP 5

Travel and other healthcare

Carry the discharge letter and treatment card, including the date and type of radioactive treatment. Ask about travel and radiation detectors. Tell any healthcare staff you have had PRRT; urgent care must never be delayed because of radiation precautions.

STEP 6

Pregnancy, breastfeeding and fertility

Lutathera can harm an unborn child and may affect fertility. Discuss fertility preservation before treatment. Use effective contraception during treatment and for seven months after the last dose for patients who could become pregnant; men with partners who could become pregnant need contraception during treatment and for four months afterwards. Breastfeeding must stop before treatment; follow specialist advice.

STEP 7

If a visit is delayed

The team decides whether a delayed administration remains suitable and whether activity must change. Do not assume a treatment can automatically proceed at the maximum interval. Continue the blood-test and symptom plan.

STEP 8

Contacts and paperwork

Write the actual NET, PRRT nursing and radiation-protection contacts on the paper plan. Example numbers are not clinical contact lines. Keep the written precautions and urgent hormone-symptom instructions with the discharge letter.

STEP 9

The Irish service’s scan appointments

The detailed Irish leaflet describes three radiation-assessment scans on separate days in the first week after the first infusion, starting the next day, and one scan after later infusions. It also describes CT or MRI between the second and third infusions and after the course. These are service arrangements to confirm on your appointment sheet, not a universal timetable for every NET.

STEP 10

Extra blood tests after the course

NCCP specifies four-weekly blood tests for at least three months, then three-monthly for the following year. The Irish service leaflet also describes fortnightly tests for eight weeks immediately after the final infusion. Your written discharge plan may therefore require more frequent early tests; follow that plan and ask the service to resolve any mismatch.

STEP 11

Paper radiation instructions for home

Before discharge, obtain a paper copy with the exact end date for each restriction. The Irish leaflet includes daily bathing, careful handwashing, sitting to pass urine and separate washing of body-fluid-soiled laundry in the first week. Ask for specific handling instructions for pads, incontinence or a catheter. Sleeping, travel, work and contact limits differ: examples include separate beds from a partner for 14 days and sleeping apart from pregnant people or children for three weeks. Your medical physicist's personal instructions take precedence.

STEP 12

Carry the official radiation card too

The medical physicist supplies a radiation-treatment card to carry for the time stated. The SACTCheck wallet card is an additional treatment record and does not replace that official card, the discharge letter or the radiation-safety plan.

Sources: NCCP 00642 v2 · current Irish Lutathera regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1

Kidney protection

A lysine/arginine infusion is required for kidney protection and begins before Lutathera. Follow the service's fluid and toileting advice. Tell staff about heart or kidney problems; do not use a generic fluid target.

2 Nausea prevention

PRRT has moderate emetogenic potential. A 5-HT3 anti-sickness medicine is given at least 30 minutes before the amino-acid infusion, with additional treatment if needed. Tell staff promptly if nausea starts.

3 Somatostatin analogue coordination

Long-acting SSA injections can interfere with receptor targeting. NCCP uses a 30-day interval before PRRT and short-acting injections are withheld for 24 hours. The referring NET consultant decides your exact stop and restart plan, balancing hormone control. Never stop injections on your own.

4 Radiation safety and practical planning

The medical physicist gives individual written and verbal advice. Contact with young children or pregnant people may need restriction for up to three weeks, and close-contact work may require up to two weeks away. Arrange transport, sleeping, toileting, laundry and caring responsibilities before admission.

Sources: nccp · smpc · hse-prrt

TREATMENT REFERENCE · 01

Your PRRT course

Receptor-directed radiation with specialist safety checks.

Why PRRT?

PRRT carries radiation to somatostatin receptors on NET cells to help control disease that cannot be removed or has spread. A specialist NET and nuclear-medicine team checks receptor imaging, organ function and your priorities before recommending it.

Sources: nccp · smpc · hse-prrt

How it works

Lutathera joins a receptor-binding molecule to the radioactive element lutetium-177. After infusion, it attaches to somatostatin receptors on suitable NET cells and delivers radiation nearby. The receptor scan helps select patients. Kidneys and blood-forming marrow can also receive radiation, which is why kidney protection and long-term blood monitoring matter.

Sources: nccp · smpc · hse-prrt

Why the amino acids matter

The amino-acid infusion reduces the amount of radioactive medicine retained in the kidneys. It is an essential part of kidney protection. It can cause nausea and affect potassium or fluid balance, so anti-sickness treatment, blood tests and observation are needed.

Sources: nccp · smpc · hse-prrt

Four intended administrations

Four administrations are planned, usually eight weeks apart. Lutathera is infused over about 30–40 minutes, with a four-hour kidney-protective amino-acid infusion and anti-sickness treatment. The visit is much longer than the Lutathera infusion; an overnight stay may be needed.

Sources: nccp · smpc · hse-prrt

Evidence and its limits

Lutetium-177 DOTATATE with standard-dose octreotide improved time before progression compared with high-dose octreotide. The final overall-survival comparison was not statistically significant. The findings do not promise a survival gain for an individual. NETTER-1 was a midgut study. Its results must not be presented as identical evidence for every pancreatic, other GI or grade 3 NET. The current Irish eligibility and specialist decision determine this regimen's use.

Sources: netter1

TREATMENT REFERENCE · 02

Preparing for hospital and returning home

The service supplies a personal written plan.

Before admission

Confirm SSTR imaging, appointments, recent bloods, hormone medicines and pregnancy precautions. Tell the team about kidney/heart problems, incontinence, a catheter, childcare or close contact with pregnant people; these may change the practical plan.

Sources: nccp · smpc · hse-prrt

Treatment day

The standard NCCP activity is 7.4 GBq by intravenous infusion, usually every eight weeks for four administrations; the specialist may alter, delay or stop treatment. Lutathera runs over approximately 30–40 minutes. A lysine/arginine amino-acid infusion starts 30 minutes beforehand and continues for four hours. An anti-sickness medicine is given at least 30 minutes before the amino acids. The first visit generally includes an overnight stay; later treatments may be day case or overnight. The service confirms your plan.

Sources: nccp · smpc · hse-prrt

SSA injections: stop and restart

The NET and nuclear-medicine teams must coordinate the injection plan. Do not translate “about four weeks” into your own date: NCCP specifies 30 days for long-acting SSA and 24 hours for short-acting SSA before PRRT. Restart timing follows the written service plan; it can differ with day-case arrangements.

Sources: nccp · smpc · hse-prrt

At home after treatment

Follow individual instructions on distance and time with other people, sleeping arrangements, toilets, body fluids, laundry, work and transport. Restrictions may extend to three weeks for some contacts. Contact the medical physicist before discharge if you cannot follow the plan at home.

Sources: nccp · smpc · hse-prrt

Travel and other healthcare

Carry the discharge letter and treatment card, including the date and type of radioactive treatment. Ask about travel and radiation detectors. Tell any healthcare staff you have had PRRT; urgent care must never be delayed because of radiation precautions.

Sources: nccp · smpc · hse-prrt

TREATMENT REFERENCE · 03

Blood tests, scans and follow-up

Tests continue between treatments and after the course.

The full safety-test sequence

Baseline blood count, kidneys/liver, ECHO/MUGA and pregnancy assessment where applicable. Counts/kidneys/liver once 2–4 weeks before and shortly before each infusion, every two weeks between, then every four weeks for at least three months after the final dose and every three months for the following year. Potassium is checked before amino acids. Renal monitoring continues at least through the first year. Post-infusion radiation scans and response scans have separate purposes.

Sources: nccp · smpc · hse-prrt

What may change treatment

The specialist may postpone, reduce or stop treatment for low counts, kidney/liver abnormalities, other toxicity or disease change. A four-stamp record shows intended administrations; it cannot confirm medical suitability.

Sources: nccp · smpc · hse-prrt

Post-treatment scans

The Irish service describes special scans after each infusion to measure radiation delivered to organs. CT/MRI or other scans to assess tumour response follow the specialist plan.

Sources: nccp · smpc · hse-prrt

Delayed effects

Kidney injury and rare bone-marrow disorders, including myelodysplastic syndrome or acute leukaemia, can occur later. Persistent tiredness, infections, bruising or bleeding needs review even after the treatment course.

Sources: nccp · smpc · hse-prrt

TREATMENT REFERENCE · 04

Urgent symptoms, hormone safety and precautions

Carry the written emergency plan.

When to seek help

Seek urgent help for sudden severe flushing with faintness, severe diarrhoea, wheeze or breathing trouble, especially in the first 48 hours; follow the hormone-crisis plan. Fever, bleeding, much less urine, persistent severe abdominal pain, muscle weakness or confusion needs prompt advice. Tell emergency staff when you received PRRT; radiation precautions must not delay urgent care.

Sources: nccp · smpc · hse-prrt

Hormone crisis

PRRT can trigger a sudden hormone release. The Irish leaflet highlights the first 48 hours as the main early-risk period. Staff monitor you and may give extra medicines or extend the stay. You need clear instructions for symptoms after discharge; a history of hormone symptoms affects planning.

Sources: nccp · smpc · hse-prrt

Tumour breakdown and fluid changes

Rapid tumour breakdown can disturb blood chemistry. New muscle cramps, weakness, confusion, palpitations, reduced urine or breathlessness needs prompt assessment. Staff also monitor potassium and fluid balance around the amino-acid infusion.

Sources: nccp · smpc · hse-prrt

Pregnancy, breastfeeding and fertility

Lutathera can harm an unborn child and may affect fertility. Discuss fertility preservation before treatment. Use effective contraception during treatment and for seven months after the last dose for patients who could become pregnant; men with partners who could become pregnant need contraception during treatment and for four months afterwards. Breastfeeding must stop before treatment; follow specialist advice.

Sources: nccp · smpc · hse-prrt

If a visit is delayed

The team decides whether a delayed administration remains suitable and whether activity must change. Do not assume a treatment can automatically proceed at the maximum interval. Continue the blood-test and symptom plan.

Sources: nccp · smpc · hse-prrt

Contacts and paperwork

Write the actual NET, PRRT nursing and radiation-protection contacts on the paper plan. Example numbers are not clinical contact lines. Keep the written precautions and urgent hormone-symptom instructions with the discharge letter.

Sources: nccp · smpc · hse-prrt

TREATMENT REFERENCE · 05

Your Irish PRRT service plan in print

Confirm bookings and restrictions with the treating service.

The Irish service's scan appointments

The detailed Irish leaflet describes three radiation-assessment scans on separate days in the first week after the first infusion, starting the next day, and one scan after later infusions. It also describes CT or MRI between the second and third infusions and after the course. These are service arrangements to confirm on your appointment sheet, not a universal timetable for every NET.

Sources: hse-prrt-long · nccp

Extra blood tests after the course

NCCP specifies four-weekly blood tests for at least three months, then three-monthly for the following year. The Irish service leaflet also describes fortnightly tests for eight weeks immediately after the final infusion. Your written discharge plan may therefore require more frequent early tests; follow that plan and ask the service to resolve any mismatch.

Sources: hse-prrt-long · nccp

Paper radiation instructions for home

Before discharge, obtain a paper copy with the exact end date for each restriction. The Irish leaflet includes daily bathing, careful handwashing, sitting to pass urine and separate washing of body-fluid-soiled laundry in the first week. Ask for specific handling instructions for pads, incontinence or a catheter. Sleeping, travel, work and contact limits differ: examples include separate beds from a partner for 14 days and sleeping apart from pregnant people or children for three weeks. Your medical physicist's personal instructions take precedence.

Sources: hse-prrt-long · nccp

Carry the official radiation card too

The medical physicist supplies a radiation-treatment card to carry for the time stated. The SACTCheck wallet card is an additional treatment record and does not replace that official card, the discharge letter or the radiation-safety plan.

Sources: hse-prrt-long · nccp

TREATMENT REFERENCE · 06

Understanding your particular NET

Several different features help the team choose treatment.

Where it started and where it is now

A neuroendocrine tumour (NET) starts in cells that receive signals and can release hormones. The original organ is the primary site. A pancreatic NET is different from the more common pancreatic adenocarcinoma. Stage describes how far the cancer has spread; the stage system depends on its original site. A tumour in the liver may be a NET that spread there from elsewhere.

Sources: nccn-patient

Differentiation, grade and Ki-67

Differentiation describes how the cells look under the microscope. Grade describes their growth activity. Ki-67 is a laboratory measure of the proportion of tumour cells showing a marker of cell growth and helps assess grade in relevant NETs. It is not the percentage of your body affected and is not a survival percentage. Ask for the exact pathology wording, grade and Ki-67 result, including the sample date.

Sources: nccn-patient

Grade is different from stage

A low-grade NET can have spread, and a high-grade tumour is not automatically stage 4. Well-differentiated grade 3 NET and poorly differentiated neuroendocrine carcinoma are different diagnoses with different treatment choices. These medicine guides must not be applied to another diagnosis simply because its name contains neuroendocrine.

Sources: nccn-patient

Functioning and nonfunctioning

A functioning NET causes a hormone-related syndrome. A nonfunctioning NET does not cause a recognisable excess-hormone syndrome, but it can still grow or cause symptoms by its size or location. Ask which description applies to you. A positive somatostatin receptor scan does not by itself mean the tumour is functioning.

Sources: nccn-patient · nci-pnet

Your diagnosis record

Keep the primary site, differentiation, grade, Ki-67 if reported, stage or sites of spread, hormone syndrome and receptor-scan result together in your passport. Ask the team to explain changes rather than trying to interpret one result in isolation.

Sources: nccn-patient

TREATMENT REFERENCE · 07

Making sense of tests and results

Safety tests, tumour tests and hormone tests answer different questions.

Blood tests for treatment safety

Blood counts measure infection-fighting cells, oxygen-carrying cells and platelets. Kidney and liver tests help the team assess how your body can handle treatment. The schedule for your medicine is shown separately. Attend these checks even when you feel well; a scan does not replace them.

Sources: nccn-patient · nccp

CT, MRI and receptor imaging

CT and MRI help show tumour size and where it is. A somatostatin receptor (SSTR) scan looks for receptor targets that certain treatments can attach to. An FDG-PET scan answers a different question about tumour activity and may be useful in selected cases. You will not necessarily need every scan. The specialist decides which test can change your care.

Sources: nccn-patient

Hormones and tumour markers are individual

The team may measure selected blood or urine markers when they answer a question about your NET or symptoms. These can include serotonin-related testing such as 5-HIAA for selected patients, or specific pancreatic hormones. Chromogranin A is not a universal test before every treatment. A changing marker alone does not prove cancer growth. Record why each test is being done, how to prepare and who will explain it.

Sources: nccn-patient · nci-net

Preparing for a test

Ask whether you need fasting, a timed urine collection, dietary preparation or a medicine review. Follow the laboratory's instructions for your particular test. Do not stop an acid-suppressing medicine, diabetes medicine or hormone-control treatment yourself. Tell the team about kidney problems, allergies and all medicines before a scan or blood test.

Sources: nccn-patient · nci-net

Heart and inherited-risk checks

A heart ultrasound may be needed for particular treatment risks or to assess heart-valve problems associated with a hormone syndrome. It is not required on the same timetable for every NET. Depending on the diagnosis, age and family history, the team may offer genetic counselling; some NETs occur with inherited conditions such as MEN1. A tumour gene test and an inherited-risk test are not interchangeable.

Sources: nccn-patient

Who does what, and when?

Use the passport to write each test, its purpose, planned date, booking service, result-review contact and next step. Your consultant sets response-scan and marker intervals. The postoperative schedules in general guides are not automatically the right schedule while you are taking these medicines.

Sources: nccn-patient · nccp

TREATMENT REFERENCE · 08

Hormone symptoms and day-to-day changes

A new symptom may come from the NET, treatment or another illness.

Build a picture of what is normal for you

Record your usual bowel pattern, flushing, pain, appetite, weight and energy before starting. Note what changes, how long it lasts, what you were taking and how it affects daily life. This helps the team distinguish a hormone problem from treatment toxicity without assuming that every symptom is cancer progression.

Sources: nccn-patient

Hormone-related symptoms

Depending on the tumour, excess hormones may cause flushing, frequent watery stools, wheezing, acid-related pain or blood-sugar disturbances. An insulin-producing tumour can cause low blood sugar. Not every NET causes these problems. Ask which symptoms matter for your diagnosis and whether you need a separate rescue plan.

Sources: nccn-patient

Hormone treatment is a separate part of the plan

Somatostatin analogue injections and other symptom treatments may continue alongside anticancer treatment when prescribed. They may have their own appointment dates. Never stop them simply because tablets are paused. PRRT requires a carefully coordinated interruption and restart plan, which the nuclear-medicine service must supply.

Sources: nccn-patient · nccp

When a change is urgent

Follow your oncology emergency plan for sudden or severe symptoms. Collapse, severe breathing difficulty, chest pain, confusion or uncontrolled bleeding needs emergency assessment. Severe diarrhoea, persistent vomiting, dehydration, fever or sudden illness needs urgent oncology advice. Do not wait for a scan, an online answer or the next clinic visit.

Sources: nccp

Procedures and hormone syndromes

Tell the surgical, dental, anaesthetic or imaging team if you have a hormone-producing NET or a previous hormone crisis. Ask your specialist whether special preparation is needed. Carry your current medicines and emergency plan; the wallet card supports this information but does not replace specialist instructions.

Sources: nccn-patient · nccp

TREATMENT REFERENCE · 09

Your overall plan and choices

Your priorities belong in the treatment plan.

What are we trying to achieve?

Ask whether the main aim is to slow growth, reduce tumour size, relieve hormone symptoms, make another treatment possible or protect day-to-day quality of life. The aim and likely benefit depend on your particular NET and previous treatment. A stable scan may represent useful control even without tumour shrinkage.

Sources: nccn-patient

How treatments fit together

Some people need surgery, hormone-control treatment, tablets, chemotherapy, PRRT, liver-directed treatment or external radiotherapy at different points. These are not all automatically part of your regimen. Ask the multidisciplinary team to draw your own sequence and name the clinician coordinating it.

Sources: nccn-patient

Choosing between options

Ask why this option suits your tumour site, grade, growth rate, symptoms, receptor status and other health conditions. Discuss alternatives, the option of observation when appropriate, supportive care, and any suitable clinical trial. A treatment listed in a US guide is not a guarantee of availability or funding in Ireland.

Sources: nccn-patient

Evidence is about groups

A trial can measure time before cancer worsens, tumour shrinkage or survival. These are different outcomes. A group result does not predict your own response or lifespan. Ask how closely the study patients match your diagnosis and what benefit would make the treatment worthwhile to you.

Sources: nccn-patient

Review points and shared decisions

Agree when you will review symptoms, tests and scans, and what would lead to continuing, delaying, changing or stopping treatment. Discuss work, caring duties, travel and the effects you most want to avoid. You can revisit your choices as your situation changes.

Sources: nccn-patient

TREATMENT REFERENCE · 10

Eating, energy, practical help and support

Supportive care can be used alongside treatment from the start.

Food, weight and digestion

Tell the team about weight loss, poor appetite, early fullness, persistent diarrhoea or greasy stools. Ask for a dietitian familiar with NETs and your surgery or medicines. Different causes need different support; do not assume every bowel symptom needs the same diet or medicine. Ask whether digestive-enzyme assessment or replacement is relevant to you.

Sources: nccn-patient · nci-pnet

Fluids and safe activity

Follow your individual fluid advice, especially with diarrhoea, kidney disease or heart problems. Balance manageable activity with rest. Tell the team if fatigue, dizziness, pain or breathlessness limits usual tasks; these symptoms can have treatable causes.

Sources: nccn-patient · nccp

Emotions, relationships and quality of life

Living with repeated tests and treatment can affect sleep, mood, relationships and confidence. Ask about psychological support, sexual health, fertility advice, symptom-control services and support for carers. Supportive or palliative care aims to improve comfort and daily life and can accompany active anticancer treatment.

Sources: nccn-patient

Practical help

Ask your nurse or social worker about transport, work, finances, home support, caring responsibilities and local NET support organisations. A general resource list may include services in other countries; your team can identify options available to you.

Sources: nccn-patient

A record that works without a phone

Keep the printed schedule, medicine list, tests, contact numbers and written emergency instructions together. The full handbook contains the information in sequence for offline reading. The QR code is an optional route to more information; it is not needed to call for help or follow your paper plan.

Sources: nccn-patient

Questions to take to the next visit

What is my exact diagnosis and grade? What is treatment trying to achieve? Which tests are mandatory for this medicine, and which depend on my NET? Who reviews results? What symptoms mean call now? What support would help me live as well as possible?

Sources: nccn-patient

What NETTER-1 did and did not show

The trial studied progressive SSTR-positive midgut NET after octreotide treatment.

Lutetium-177 DOTATATE with standard-dose octreotide improved time before progression compared with high-dose octreotide.

Sources: netter1

PREPARE FOR THE CONVERSATION

Questions worth asking

Does my receptor scan and exact NET diagnosis meet this PRRT indication?

The specialist team must confirm progressive, well-differentiated G1/G2 SSTR-positive GEP-NET and assess other eligibility factors. A receptor scan alone does not establish that treatment is suitable.

Will the first or later visits require overnight admission?

The Irish leaflet describes an overnight stay for the first treatment; later treatments may be day case or overnight. Confirm the actual admission and discharge plan with the PRRT service.

Who provides the written stop/restart dates for my SSA injections?

The referring NET consultant and nuclear-medicine team coordinate the injection plan. Get exact written dates; do not stop or restart an SSA on your own.

When are my fortnightly and post-course blood tests?

NCCP specifies two-weekly bloods between administrations and continued tests after the course. The Irish service may arrange additional early post-course checks. Ask for one reconciled written schedule and result-review contact.

Which household, work and travel precautions apply, and on what dates do they end?

The medical physicist gives individual restrictions, which can extend to three weeks for some contacts. Obtain each end date and discuss transport, sleeping, childcare and work before admission.

What symptoms suggest a hormone crisis, and who do I call after discharge?

Sudden severe flushing, faintness, diarrhoea, wheeze or breathing trouble can require urgent assessment, particularly in the first 48 hours. Follow the service's written emergency plan and do not delay urgent care because of radiation precautions.

What fertility, contraception and long-term follow-up plan do I need?

Discuss fertility before starting. Current EMA/HSE information requires contraception during treatment and for seven months afterwards for patients who could become pregnant, and four months for men with partners who could become pregnant. Blood and organ-function follow-up continues after the fourth administration.

> **Try explaining the plan back:** Can you explain the four planned visits, your blood-test timetable, the separate SSA injection plan, radiation precautions and who to call for sudden hormone symptoms?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00642 v2. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00642 v2 · current Irish Lutathera regimen
- EMA Lutathera product information and patient leaflet · updated 14 August 2026
- HSE / Irish PRRT service: introductory patient information
- NCCN Guidelines for Patients: Neuroendocrine Tumors, 2026 · education reference
- NCI PDQ: gastrointestinal NETs · selected marker background
- NCI PDQ: pancreatic NETs · hormone and digestive background
- NETTER-1 final overall-survival and long-term safety report
- HSE / Irish PRRT service: detailed patient information

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-05.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6683/642_v2_Lutetium_Lu177_oxodotretide.pdf

https://www.ema.europa.eu/en/documents/product-information/lutathera-epar-product-information_en.pdf

https://assets.hse.ie/media/documents/Neuroendocrine_PRRT_Patient_Information_Leaflet_short.docx

https://www.nccn.org/patients/guidelines/content/PDF/neuroendocrine-patient.pdf

https://www.cancer.gov/types/gi-neuroendocrine-tumors/hp/gi-neuroendocrine-treatment-pdq

https://www.cancer.gov/types/pancreatic/hp/pnet-treatment-pdq

https://pubmed.ncbi.nlm.nih.gov/34793718/

https://assets.hse.ie/media/documents/Neuroendocrine_PRRT_Patient_Information_Leaflet_long.docx